

Getting Ready for Your Third Guided AI Session

A short guide to read before you begin

This guide explains what will happen in your third session with the AI assistant. Reading it first means you'll know what to expect, why the session is shaped the way it is, and how to get the most out of it. It takes just a few minutes to read.

Welcome back — from the big picture to a single thought

In your first session, you looked at how the mind works moment to moment: a situation sparks an automatic thought, the thought stirs an emotion, the emotion shapes what you do and how your body reacts — and it all loops back on itself. You also met the cognitive distortions. In your second session, you stepped back and mapped the bigger picture — the problems you'd most like to be free from, and the goals you'd like to move toward.

This session brings those two views together and adds the tool at the very heart of TBCT. You'll take one real, distressing thought — the kind that feeds the problems you mapped last time — and examine it slowly and fairly, using a simple set of questions called the **Intrapersonal Thought Record**, or **Intra-TR**. By the end, the aim is a fairer, more balanced way of seeing the situation — and, often, a lighter feeling to go with it.

Good to know: There are no right or wrong answers here, and nothing you say is being graded. The Intra-TR simply helps you look at one thought from several angles — the way you might turn an object over in your hands to see it properly.

What this session is

You'll work through the Intra-TR with the AI guide — a set of **fourteen plain questions** that carry a single thought from “here's what's bothering me” all the way to “here's a fairer conclusion, and here's what I'll do next.” You'll apply it to one real situation from your own life, chosen together, and gentle enough to learn on.

The fourteen questions are printed in the **Annex** at the end of this guide, so you can see the whole path before you start.

Who you'll be talking to

As before, you'll be chatting with an AI assistant set up to guide this one exercise — a friendly companion for this particular step. A few things to keep in mind:

- It is **not a replacement** for your therapist or doctor. It's guiding one exercise, not managing your overall care.

- It won't tell you what to think, or what your conclusion “should” be. The questions belong to the guide; the answers are always yours.
- It stays gently focused on this exercise. If you raise something else, it will kindly keep to the task and suggest saving that for your therapist.

How the session will flow

The session moves through the fourteen questions in order, at an unhurried pace. You don't need to memorise them — the guide leads the way — but here's the shape, so nothing feels out of the blue.

Part 1 — Describing the moment

You'll start by naming a **specific situation** and the **automatic thought** that popped up in it. Then you'll notice the **emotion** it stirred and how strong it was, **what you did** (or wanted to do), and **what happened in your body**. This is the same cycle you met in your first session — now written down one piece at a time.

Part 2 — Turning the thought over

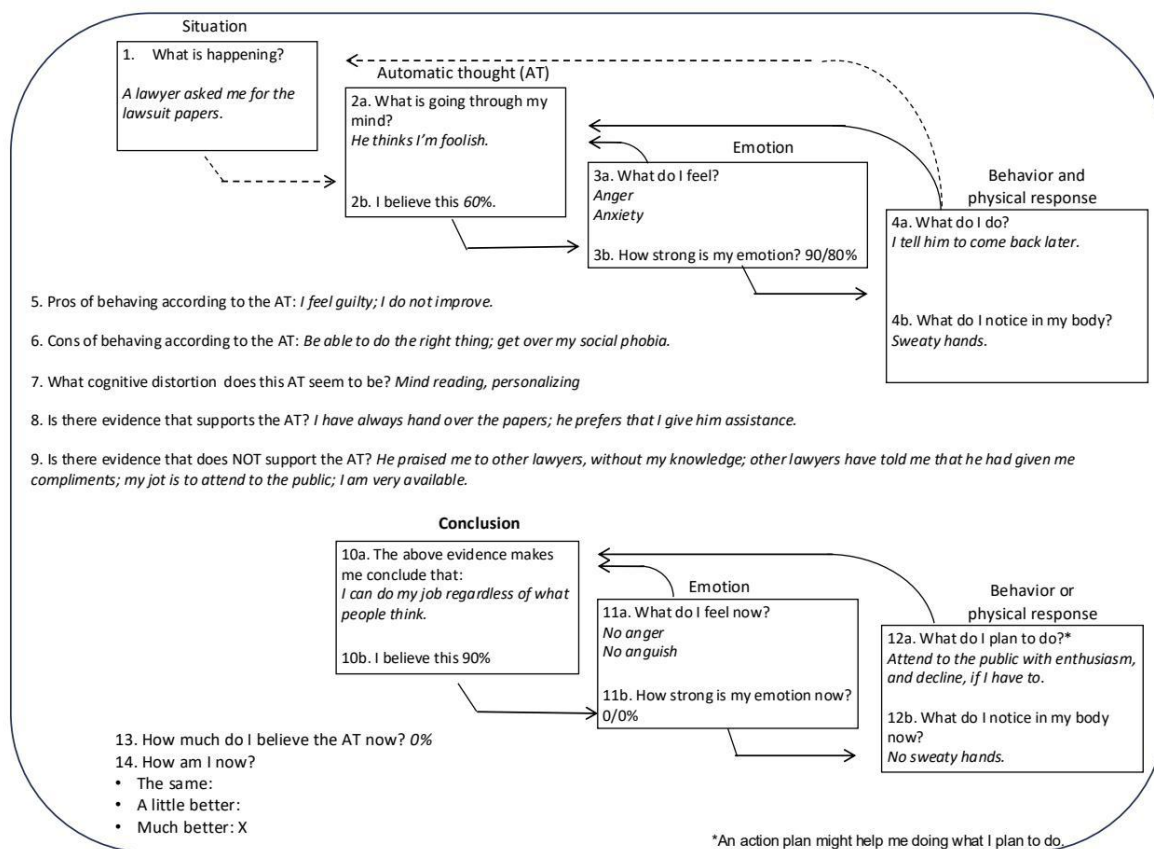
Next you'll weigh the thought itself. You'll look briefly at the **pros and cons** of reacting the way you did, check the thought against your **cognitive distortions list** from Session 1, and then — this is the heart of it — gather the **evidence for** the thought and the **evidence against** it, as fairly as a good friend would.

The most useful question: Many people find the turning point is the evidence that does not support the thought — the facts, recent moments, or things you've actually done that quietly contradict it. It's worth taking your time here.

Part 3 — Reaching a fairer conclusion

With both sides in view, you'll draw your **own balanced conclusion** — what the full picture really suggests — and notice how much you believe it. Then you'll check what you **feel now**, what you **intend to do** next, and how your **body** feels compared to the start. Finally, you'll rate how much you still believe the original thought, and how you're doing overall.

TBCT Intrapersonal Thought Record (IntraTR)



A completed Intra-TR, shown here with an example. Copyright: Irismar Reis de Oliveira; trial-basedcognitivetherapy.com.

Seeing a finished one can make the path clearer — but yours will be about **your own** situation, in your own words.

What the conversation will feel like

- **Lots of questions.** The guide leads mostly by asking, not lecturing. This is on purpose — the insights land better when they're yours.
- **One step at a time.** You'll answer one question before moving to the next, so nothing piles up.
- **Unhurried.** There's no clock, and no need to have polished answers ready.
- **Okay to be unsure.** If a question feels hard or abstract, just say so — the guide will slow down or try another angle.
- **Conversational.** It's meant to feel like talking with a knowledgeable friend, not filling in a form.

What to have ready before you start

1. **The Intra-TR worksheet.** The fourteen questions are printed in the Annex — have it open or nearby. The guide will check you have it before you begin.
2. **Your cognitive distortions list from Session 1.** You'll use it for one of the questions, so keep it within reach.

3. **A real but manageable example.** A situation that genuinely bothered you — real, but not the most painful thing you can think of. A moderate, everyday example works best for learning the method.
4. **A quiet moment.** Set aside a stretch of uninterrupted time where you can focus and reply thoughtfully.
5. **An open, curious frame of mind.** You're here to look honestly at one thought — not to judge yourself.

A few tips to get the most from it

- Answer honestly — the exercise only reflects back what you put into it.
- It's fine to think out loud. Half-formed answers are useful too.
- When you look for evidence against the thought, cast a wide net: your general health, recent things you've done, times you've coped before, feedback from others, plain facts.
- Afterwards, keep your finished Intra-TR and, if you can, share it with your therapist at your next appointment.

What this session is not

So there are no surprises, here's what the AI **won't** do:

- It won't diagnose you or manage your treatment.
- It won't provide crisis support or emergency care.
- It won't tell you what to conclude, feel, or do — those are always yours.
- It won't push you toward other techniques; it stays within this one exercise.

If you're in distress This session is a learning exercise, not an emergency service. If at any point you feel overwhelmed, or you have thoughts of harming yourself, please pause and reach out to your study clinician or your local emergency services right away. Your safety matters more than finishing the exercise.

You don't need to prepare anything else — the AI will guide you step by step. When you're ready, begin whenever you like.

Your Intra-TR Worksheet

Keep this nearby during your session. These are the fourteen questions your guide will walk you through, in order — read them over beforehand so the path feels familiar. The blank form below shows how the pieces fit together.

TBCT Intrapersonal Thought Record (IntraTR)

Situation

1. What is happening?

Automatic thought (AT)

2a. What is going through my mind?

2b. I believe this _____ %

Emotion

3a. What emotion do I feel?

3b. How strong is it? _____ %

Behavioral and physical response

4a. What do I do?

4b. What do I notice in my body?

5. Pros of behaving according to the AT: _____

6. Cons of behaving according to the AT: _____

7. What cognitive distortion does this AT seem to be? _____

8. Is there evidence that supports the AT? _____

9. Is there evidence that does NOT support the AT? _____

Conclusion

10a. The above evidence makes me conclude that: _____

10b. I believe this _____ %

11a. What emotions do I feel now?

Positive: _____ %

Negative**: _____ %

11b. How strong are they?

Positive: _____ %

Negative**: _____ %

12a. What do I intend to do?*

12b. What do I notice in my body now?

13. How much do I believe the AT now? _____ %

14. How am I now?

- The same: ☐
- A little better: ☐
- Much better: ☐

*An action plan (Session 4) might help perform this intention.
**The negative emotion elicited by the AT is reassessed here.

The TBCT Intrapersonal Thought Record (Intra-TR). Copyright: Irismar Reis de Oliveira; trial-basedcognitivetherapy.com.

Where it fits	What the guide will ask
Situation	1. What is happening? (Describe it as if it were happening right now.)
Automatic thought	2a. What is going through my mind? 2b. How much do I believe it, from 0 to 100%?
Emotion	3a. What emotion do I feel? 3b. How strong is it, from 0 to 100%?
Behaviour & body	4a. What do I do (or want to do)? 4b. What do I notice in my body?
Pros & cons	5. Pros of reacting this way — even a moment's relief? 6. Cons of reacting this way?

Where it fits	What the guide will ask
Distortion	7. Which cognitive distortion does the thought seem to be? (Use your Session 1 list.)
Evidence	8. Is there evidence that supports the thought? 9. Is there evidence that does NOT support it?
Conclusion	10a. Taking it all together, what do I conclude? 10b. How much do I believe the conclusion (0–100%)?
New emotion	11a. What do I feel now — anything positive, and the earlier feeling? 11b. How strong are they (0–100%)?
New intention & body	12a. What do I intend to do now? 12b. What do I notice in my body now?
Final check	13. How much do I believe the original thought now (0–100%)? 14. How am I now — the same, a little better, or much better?

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